

chondrosarcoma-mets-acetabular-fx-id h-unknown-y34r

Standard of care options — approved and guideline-endorsed strategies

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Standard of care options

The treatment strategies that are standard for this patient's situation, meaning a regulator approved them for a population that includes this patient or a major academic or clinical-society guideline carries them. This report runs alongside the targetable-feature ranking rather than in place of it, and it does not narrow what that ranking surfaces.

Assessed: 9 standard options. **Consider now:** 6. **Behind an open gate:** 1. **Already received:** 0.

Nine standard options were screened for this treatment-naive metastatic dedifferentiated chondrosarcoma, and six can be weighed now. The guideline-carried core is short: doxorubicin-based chemotherapy, which holds the only on-label footing here (the FDA doxorubicin label names metastatic bone sarcomas) and whose first-line response expectation is honestly contested, 9% in a modern multi-institutional cohort against 20.5% in an older locally-assessed series; orthopaedic fixation of the fractured acetabulum, which is also the case's sequencing keystone, since irradiated bone heals poorly and any radiotherapy to that field waits on the fixation call; palliative radiotherapy itself; and pre-treatment referral to a high-volume sarcoma centre, on survival evidence that is real but drawn almost entirely from soft-tissue sarcoma networks. The best-evidenced single item on this page is early specialist palliative care alongside active treatment, a strong ASCO recommendation resting on randomised data, and it costs no line of therapy. Trial enrollment is itself guideline-carried management in this disease and matches the one preference the patient actually stated.

Nothing standard has been spent: there is no prior therapy on file.

One option sits behind a gate. Ivosidenib is carried by NCCN for IDH-mutant dedifferentiated disease but waits on the IDH1 sequencing the ranked workup already orders, and its FDA label does not yet include chondrosarcoma. Pazopanib and pulmonary metastasectomy were assessed and set aside because their guideline populations, conventional histology and oligometastatic lungs, are not hers.

This report is additive. It supplies the standard options and does not narrow, reorder, or filter the experimental options ranked elsewhere on this case page; where the two tracks share a line, the rows name the trade instead of resolving it.

Options to consider now

Standard options this patient is eligible for, with an endorsement that covers their situation, and no unresolved gate in front of them.

Doxorubicin-based chemotherapy (doxorubicin alone or with ifosfamide or cisplatin)

Priority: essential **Modality:** Systemic chemotherapy **Line:** First line **Intent:** disease-control intent

What makes it standard.

- FDA On-label approval (generic doxorubicin hydrochloride injection) (covers this patient · Label revised 2024-07)
 - covers: Doxorubicin hydrochloride injection is indicated for the treatment of: acute lymphoblastic leukemia, acute myeloblastic leukemia, Hodgkin lymphoma, non-Hodgkin lymphoma, metastatic breast cancer, metastatic Wilms' tumor, metastatic neuroblastoma, metastatic soft tissue sarcoma, metastatic bone

sarcomas, and other listed carcinomas.

- NCCN Chondrosarcoma pathway; dedifferentiated disease follows the osteosarcoma systemic-therapy algorithm (category not re-verified this run; guideline login-walled) (partial population match · NCCN Bone Cancer v2.2025)
 - covers: Dedifferentiated chondrosarcoma: systemic therapy per the osteosarcoma pathway (doxorubicin- and cisplatin-based regimens). Conventional chondrosarcoma is treated as not meaningfully chemosensitive.
- other_society British Sarcoma Group UK guideline statement: osteosarcoma chemotherapy protocols 'can be considered' (covers this patient · Br J Cancer 2025;132:32-48)
 - covers: 'Although the role of chemotherapy in dedifferentiated chondrosarcoma is not well defined, osteosarcoma chemotherapy protocols can be considered as neo-adjuvant and or adjuvant therapy although survival is unfortunately poor.' For advanced disease: 'Chemotherapy is of limited benefit in metastatic mesenchymal or dedifferentiated chondrosarcoma.'

Fit to this patient. Conditionally eligible prior_therapies[] is empty, so the first-line window is intact; what is missing is every gating measurement: no baseline echocardiogram before an anthracycline, no renal or hepatic panel, and the ECOG of 2 is an intake assumption rather than a measured value.

- blocker: Baseline LVEF not obtained; an anthracycline cannot start without it
- blocker: Renal and hepatic panels not obtained (also gates ifosfamide or cisplatin dosing)
- blocker: ECOG 2 is assumed, not measured; fitness decides between soft-tissue-sarcoma dosing and osteosarcoma-type intensification
- blocker: Expert bone-sarcoma pathology review of the dedifferentiated component is still a hardening item on the histology this fork rests on

The one systemic option with an on-label footing for this exact situation (the FDA doxorubicin label names metastatic bone sarcomas) and with guideline carriage for the dedifferentiated variant, the histology in which the chemotherapy fork is answered in the affirmative. The expected first-line response is contested rather than settled: 9% in the modern multi-institutional cohort against 20.5% in a 1988-2011 series read by local assessment, and the EURO-B.O.S.S. metastatic 29% is a 23-patient fitness-selected uncontrolled subgroup at $p=0.12$ whose only significant split tracks surgical remission. Dosing intensity, standard soft-tissue-sarcoma-type doxorubicin versus osteosarcoma-type intensification, was left open by the board and turns on the fitness and organ-function measurements that have not yet been made.

Key evidence.

- Bui 2023 multi-institutional DDCS cohort · retrospective multi-institutional cohort · $n=74$ · First-line ORR 9% despite mostly osteosarcoma-protocol-style regimens; metastatic median OS 7.2 months, 0% alive at 5 years
- Italiano 2013, French Sarcoma Group · multicentre retrospective cohort, 1988-2011 · $n=180$ · Dedifferentiated ORR 20.5% by local assessment; the $p=0.04$ attached to that figure is a between-subtype comparison, not a treatment effect
- EURO-B.O.S.S. dedifferentiated cohort · prospective non-randomised multicentre study, no comparator · $n=57$ · 5-year OS 29% in the 23-patient metastatic subgroup ($p=0.12$, uncontrolled); 18 of 57 never completed 6 of the 9 planned cycles; the one significant split is surgical remission, 49% vs 23% ($p=0.004$), so the survival tail may track resectability rather than the regimen

Toxicities that would change the decision. Anthracycline cardiotoxicity; LVEF monitoring before and during treatment; Myelosuppression and febrile neutropenia, the main deliverability risk at an unmeasured performance status; Ifosfamide neurotoxicity and nephrotoxicity, cisplatin nephro- and ototoxicity, if a two-drug regimen is

chosen

Alongside the targeted options. Competes for the same line of therapy. First-line chemotherapy and the IACS-6274 phase 1 seat occupy the same window: that protocol has no prior-therapy requirement, and the board noted a first-line dose-escalation slot would spend the one line with a proven response signal in this histology. Several ranked options (abemaciclib on NCT04040205, TAPUR, sunitinib plus nivolumab) are pre-positioned for the line after an anthracycline, so receiving it is also what opens them. The trade is named here, not resolved.

Last verified. 2026-08-23

References. pmid:37174084; pmid:24099780; pmid:33990016; pmid:40203873; pmid:39550489; fda:DailyMed setid 1fd148fb-0fbc-4b6f-b790-23546fb46a71

Early referral to a specialist interdisciplinary palliative care team, alongside active treatment

Priority: essential **Modality:** Supportive care **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- ASCO Guideline recommendation (2024 update): refer early, concurrent with active treatment (covers this patient · J Clin Oncol 2024;42:2336-2357)
 - covers: 'Oncology clinicians should refer patients with advanced solid tumors and hematologic malignancies to specialized interdisciplinary palliative care teams that provide outpatient and inpatient care beginning early in the course of the disease, alongside active treatment of their cancer.' The update also notes patients on phase I trials may be referred.

Fit to this patient. Eligible Advanced solid tumour with a symptomatic skeletal lesion; nothing in the profile gates a palliative care referral, and the undocumented pain and weight-bearing status is itself a reason to involve the team early.

The strongest-evidenced recommendation on this page: a current ASCO guideline, resting on randomised trials, that says early specialist palliative care belongs alongside active treatment in advanced cancer, exactly this situation. It costs no line of therapy, closes no trial door, and addresses the pain and function problems of a pathologic acetabular fracture that the record has not yet even quantified. The ASCO update also covers patients enrolled on phase I trials, so it is compatible with the stated preference to pursue every therapeutic option.

Key evidence.

- Temel 2010 · randomised controlled trial, single centre · n=151 · Better quality of life at 12 weeks (FACT-L 98.0 vs 91.5, p=0.03), less aggressive end-of-life care, and median OS 11.6 vs 8.9 months (p=0.02)

Alongside the targeted options. Can run alongside the targeted options. Runs alongside every ranked option and forecloses none. The board's critic observed that this recommendation is the best-evidenced single item put in front of the board, and it carries randomised data where the rest of the case rests on retrospective cohorts.

Last verified. 2026-08-23

References. pmid:38748941; pmid:20818875

Referral to a high-volume sarcoma centre with a bone-sarcoma multidisciplinary team, before first treatment

Priority: essential **Modality:** Other **Line:** Not line-indexed **Intent:** life-prolonging intent

What makes it standard.

- other_society British Sarcoma Group UK guideline: surgery by a bone sarcoma MDT surgeon in a commissioned centre (covers this patient · Br J Cancer 2025;132:32-48)
 - covers: 'Patients should undergo definitive resection of their sarcoma by a surgeon who is a member of a bone sarcoma MDT in a commissioned centre or if more appropriate, by a surgeon with tumour site-specific or age-appropriate skills, in consultation with the bone sarcoma MDT.'
- ESMO CPG recommendation on management at reference centres (level/grade not re-verified this run; full text paywalled) (covers this patient · Ann Oncol 2021;32:1520-1536)
 - covers: Bone sarcoma diagnosis and treatment should be carried out at reference centres or within networks with multidisciplinary bone-sarcoma expertise, from biopsy onward.

Fit to this patient. Eligible Treatment-naïve, so the pre-treatment referral window the NETSARC data speak to is still open. Geography is not recorded in the profile (geography_band null), so travel burden is the one unassessed variable.

Every society that writes bone-sarcoma guidance places management of a case like this in a specialist centre with a bone-sarcoma MDT, and the referral matters most now, before first treatment, which is the window the NETSARC data describe. The honest caveat carried from the board: both NETSARC analyses are overwhelmingly soft-tissue sarcoma, so the survival association is extrapolated to a bone primary rather than measured in one. The fixation decision, the pathology hardening review, and the trial map all become easier to run well from inside such a centre.

Key evidence.

- NETSARC pre-treatment MDT presentation · nationwide prospective registry analysis · n=12528 · Presentation to a specialist MDT before first treatment was associated with better guideline compliance (biopsy before surgery, quality of surgery, fewer reoperations, all $p<0.001$) and better local relapse-free and relapse-free survival in multivariate analysis
- NETSARC surgery in reference centres · nationwide prospective registry analysis · n=35784 · Surgery in a NETSARC reference centre was independently associated with overall survival, HR 0.681 (95% CI 0.618-0.749), and with local relapse-free and event-free survival (all $p<0.001$)

Alongside the targeted options. Can run alongside the targeted options. An expert centre is also where the ranked workup (comprehensive DNA plus RNA profiling on the dedifferentiated component) and most protocol seats are reachable, so this option widens rather than competes with the experimental track.

Last verified. 2026-08-23

References. pmid:39550489; pmid:34500044; pmid:29117335; pmid:31081028

Clinical trial enrollment as a management strategy

Priority: high **Modality:** Other **Line:** Any line **Intent:** disease-control intent

What makes it standard.

- NCCN Standing statement carried on every NCCN guideline: 'NCCN believes that the best management of any patient with cancer is in a clinical trial. Participation in clinical trials is especially encouraged.' (covers this patient · NCCN Bone Cancer v2.2025)
 - covers: All patients with cancer, all settings; printed as a standing principle across the NCCN Guidelines including Bone Cancer.
- other_society British Sarcoma Group UK guideline: trial approaches named for advanced chondrosarcoma (covers this patient · Br J Cancer 2025;132:32-48)
 - covers: 'Inoperable, locally advanced and metastatic chondrosarcomas have a poor prognosis... Other approaches in clinical trials include immunotherapy, IDH1 inhibitors and DR5 agonists.'

Fit to this patient. Conditionally eligible prefers_trials is true and no therapy has been used, but the ECOG of 2 is an intake assumption and most protocols require a measured 0-1 or 0-2; the molecular profile that most genotype-matched protocols screen on is entirely unmeasured.

- blocker: ECOG unmeasured; the assumed 2 sits at or past the cutoff of most protocols
- blocker: No comprehensive molecular profile yet; genotype-gated seats cannot be screened for until the ranked workup returns

Trial participation is itself guideline-carried management in this disease: NCCN's standing statement makes it the preferred frame for any cancer patient, and the UK guideline names trial approaches as the forward path in metastatic chondrosarcoma, where standard systemic options are thin. For this patient it also matches the one preference she actually stated, pursuing all possible therapeutic options. The gating facts are a measured performance status and the pending molecular profile, both already flagged elsewhere on this page.

Alongside the targeted options. Can run alongside the targeted options. The Experimental table on this case page is the concrete instantiation of this row: it holds the ranked protocol seats. This row records only that guideline text itself points to trials in metastatic chondrosarcoma; it does not rank among them or substitute for them.

Last verified. 2026-08-23

References. pmid:40203873; pmid:39550489

Orthopaedic stabilisation (fixation) of the pathologic acetabular fracture

Priority: high **Modality:** Surgery **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- NCCN Chondrosarcoma pathway carries surgical management for local control and palliation in metastatic disease (category not re-verified this run; guideline login-walled) (partial population match · NCCN Bone Cancer v2.2025)
 - covers: Recurrent or widely metastatic chondrosarcoma: surgical management, radiotherapy, or best supportive care are among the options for local disease control and symptom palliation. The pathway does not separately adjudicate fixation versus oncologic resection.
- ASTRO Recommendation: patients with nonspine bone metastases requiring surgery are recommended postoperative RT (different population · Pract Radiat Oncol 2024;14:377-397)

- covers: Palliative management of symptomatic bone metastases: for lesions requiring surgery, operate first and irradiate postoperatively. Written for bone metastases from other primaries, not for a primary bone sarcoma; carried here for the surgery-then-radiotherapy ordering principle.

Fit to this patient. Conditionally eligible The profile documents neither surgical fitness (no cardiac, renal, or hepatic baseline) nor the pain and weight-bearing status that defines the indication; the assumed ECOG of 2 was itself derived from this fracture, so the operation and the fitness measurement are entangled.

- blocker: Surgical fitness not assessed; no organ-function baseline on file
- blocker: Pain and weight-bearing status not documented, which bears on whether fixation is needed at all and on its urgency relative to systemic therapy

The operation on the table is fixation of a fractured weight-bearing joint, a structural and palliative decision, and not oncologic resection of the primary; the board explicitly separated the two after noting that resection had been inheriting fixation's justification, and the SEER subgroup gives resection no survival association in her histology. Fixation is also the sequencing keystone: it precedes any radiotherapy to the field, and it may change the measured performance status on which chemotherapy dosing and trial eligibility both hang. Guideline carriage for the metastatic setting is generic local-control language rather than a fixation-specific recommendation, which is why this row sits at high rather than essential.

Key evidence.

- Song 2019, SEER (resection caveat) · retrospective population-based cohort (SEER) · n=200 · Resection of the primary showed no survival association in the dedifferentiated and high-grade subgroup, an interaction analysis in 200 patients: it withdraws the oncologic benefit claim for resection without proving futility. The structural case for fixation is a separate question this literature does not answer
- Radiation and fracture healing, rat femoral model · controlled preclinical animal study · Irradiated fractures healed as atrophic non-unions; the basis of the fixation-before-radiotherapy sequencing constraint two board personas co-signed

Toxicities that would change the decision. Major pelvic surgery in a patient whose fitness has not been measured; perioperative risk is currently unquantifiable; Surgical recovery time competes with the fast tempo of metastatic dedifferentiated disease and could delay systemic therapy

Alongside the targeted options. Comes before the targeted options. Fixation before any irradiation of the fracture site is the standing sequencing constraint on this case (irradiated bone forms atrophic non-unions in the preclinical model), which reaches the ranked particle-therapy row and this table's palliative radiotherapy row alike. Whether fixation improves or spends the measured ECOG also bears on the ECOG 0-1 gates of the ranked trial seats; the board split on that and the split is preserved, not resolved.

Last verified. 2026-08-23

References. pmid:30762691; pmid:23606416; pmid:40203873; pmid:38788923; pmid:17720491

Palliative radiotherapy to the acetabular lesion

Priority: high **Modality:** Radiotherapy **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- other_society British Sarcoma Group UK guideline: radiotherapy may be offered for palliation (covers this patient · Br J Cancer 2025;132:32-48)
 - covers: 'Radiotherapy may be offered for unresectable disease, adjuvantly after surgery for close or positive margins, and for palliation.' And across bone sarcoma types: 'Radiotherapy has a palliative role in all tumour types.'
- ASTRO Recommendation: RT for pain from bone metastases; conventional schedules 800 cGy/1 fx, 2000 cGy/5 fx, 2400 cGy/6 fx, or 3000 cGy/10 fx (different population · Pract Radiat Oncol 2024;14:377-397)
 - covers: Palliative external beam radiotherapy for symptomatic bone metastases. Written for metastatic deposits from other primaries; this lesion is a primary bone sarcoma, so the schedule menu transfers more safely than the efficacy expectation.

Fit to this patient. Likely eligible No prior radiotherapy and no contraindication on file. The open question is sequencing rather than eligibility: the fixation decision comes first because irradiated bone heals poorly.

Guidelines carry palliative radiotherapy for exactly this problem, a painful unresectable skeletal lesion in metastatic disease, and the UK guideline names palliation as a radiotherapy role in every bone sarcoma type. The load-bearing caveat is ordering, not indication: the fixation decision precedes any irradiation of the fracture site, because irradiated bone forms atrophic non-unions. The efficacy expectation should be set against chondrosarcoma's relative radioresistance rather than against the bone-metastasis literature the dose schedules come from.

Key evidence.

- Radiation and fracture healing, rat femoral model · controlled preclinical animal study · Irradiated fractures healed as atrophic non-unions; radiating a fractured pelvis before the fixation question is settled risks a non-union

Toxicities that would change the decision. Impaired fracture healing if delivered before fixation is decided; the standing sequencing constraint on this case; Pelvic fields carry bowel, bladder, and marrow dose; conventional chondrosarcoma is relatively radioresistant, though the dedifferentiated component is the more radioresponsive part

Alongside the targeted options. May foreclose a targeted option. Photon radiotherapy to the acetabulum would complicate any later particle-therapy course to the same field. The ranked particle-therapy row is currently not enrollable for her (both protocol routes exclude distant metastases), so the overlap is named for completeness rather than as a live conflict.

Last verified. 2026-08-23

References. pmid:39550489; pmid:38788923; pmid:23606416

Standard options behind an open gate

Endorsed for this patient's situation, but gated on a result that is missing, pending, or below threshold. The gate has to close before the option is real.

Ivosidenib, conditional on an IDH1 mutation being found

Priority: medium **Modality:** Targeted therapy **Line:** Any line **Intent:** disease-control intent

What makes it standard.

- NCCN IDH inhibitors carried for dedifferentiated chondrosarcoma with IDH mutations (carriage confirmed via a secondary citation of v2.2025; category not re-verified this run) (population match unclear · NCCN Bone Cancer v2.2025)
 - covers: Dedifferentiated chondrosarcoma harbouring an IDH mutation; IDH inhibitors named among systemic options. The patient's IDH1/IDH2 status is untested, so whether she is in this population is unknown.

Fit to this patient. Conditionally eligible Wholly conditional on an IDH1 mutation call that has not been made: IDH1/IDH2 are untested in the profile, and untested means unassessable, not negative. Pre-test probability in dedifferentiated disease is high (roughly 50-71% across series), which is why the gate is worth closing.

- blocker: IDH1 mutation status untested; the option does not exist for her until tumour sequencing returns a susceptible mutation

Biomarker gate. IDH1 mutation (codon 132) Susceptible IDH1 mutation detected in tumour tissue (never measured)

NCCN's current Bone Cancer guideline carries IDH inhibitors for IDH-mutant dedifferentiated chondrosarcoma, which by regulatory maturity earns ivosidenib a row here even though its FDA label (revised 2024-07) still lists only AML, MDS, and cholangiocarcinoma. The gate is open: IDH1 is untested, and the tissue sequencing the experimental track ranks first is what closes it. The efficacy expectation should be read honestly by histology: the durable-control signal is in conventional chondrosarcoma, while the one published dedifferentiated-specific IDH-inhibitor figure is a median PFS of 1.5 months with a different agent.

Key evidence.

- Ivosidenib phase 1, long-term follow-up · phase 1 dose-escalation and expansion, long-term follow-up · n=21 · Conventional-histology ORR 23.1%, median duration of response 53.5 months, median PFS 7.4 months; no dedifferentiated-specific response is reported
- Chemotherapy in dedifferentiated chondrosarcoma, 2026 review · narrative review · Carries the only published IDH-inhibitor efficacy figure in dedifferentiated disease: olutasidenib median PFS 1.5 months

Toxicities that would change the decision. Generally manageable in the chondrosarcoma phase 1; QT prolongation is the label-level watch item

Alongside the targeted options. Competes for the same line of therapy. This is the same agent the Experimental ranking carries at rank 13, behind the same untested IDH1 gate that the rank-1 profiling row exists to close. This row adds its guideline footing and its regulatory limits (no chondrosarcoma indication on the FDA label; the phase 3 route, CHONQUER NCT06127407, enrolls conventional histology only); it is one option viewed from two tables, not two options.

Last verified. 2026-08-23

References. pmid:40203873; pmid:40100120; pmid:41895355; pmid:32208957; nct:NCT06127407; fda:DailyMed setid 65d254c0-67ad-42c4-b972-ad463b755b2d

Assessed and set aside

Screened against this patient and found not to apply. Listed for the audit trail.

Pazopanib (carried for the conventional-chondrosarcoma population, which this is stated not to be)

Priority: low **Modality:** Targeted therapy **Line:** Any line **Intent:** disease-control intent

What makes it standard.

- NCCN Chondrosarcoma systemic-therapy option for conventional histology (category not re-verified this run; guideline login-walled) (different population · NCCN Bone Cancer v2.2025)
 - covers: Unresectable or metastatic conventional chondrosarcoma. The supporting phase 2 explicitly excluded dedifferentiated histology.
- other_society British Sarcoma Group UK guideline: activity statement in conventional disease (different population · Br J Cancer 2025;132:32-48)
 - covers: 'Pazopanib has demonstrated activity in conventional chondrosarcoma.'

Fit to this patient. Likely ineligible The stated histology is high-grade with dedifferentiated features; the pazopanib signal and its guideline carriage are specific to conventional chondrosarcoma, and the phase 2 excluded dedifferentiated subtypes.

- blocker: Histology mismatch: carriage and evidence are for conventional chondrosarcoma; dedifferentiated disease was excluded from the supporting trial

Carried so the chondrosarcoma pathway reads complete rather than because it fits: pazopanib's guideline position and its phase 2 evidence are in conventional chondrosarcoma, and that trial excluded dedifferentiated histology. It would only enter consideration if the pending expert bone-sarcoma pathology review reclassified the tumour, and that review is a hardening item, not an open fork.

Key evidence.

- Chow 2020 phase 2 · single-arm multicentre phase 2 · n=47 · DCR at 16 weeks 43% (95% CI 28-58), 2-sided p=0.09 against a 30% null; one partial response; median PFS 7.9 months (95% CI 3.7-12.6); median OS 17.6 months (95% CI 11.3-35.0)

Toxicities that would change the decision. Hypertension (grade 3+ in 26% of the phase 2) and transaminase elevation

Alongside the targeted options. Independent of the targeted options.

Last verified. 2026-08-23

References. pmid:31509242; pmid:40203873; pmid:39550489

Pulmonary metastasectomy or local ablation of lung metastases

Priority: low **Modality:** Surgery **Line:** Not line-indexed **Intent:** disease-control intent

What makes it standard.

- other_society British Sarcoma Group UK guideline: consider for oligometastatic pulmonary disease (different population · Br J Cancer 2025;132:32-48)
 - covers: 'Surgery or local ablation should be considered for oligometastatic pulmonary disease.' General principle: 'Pulmonary metastatectomy may be indicated for oligometastatic disease, aiming to remove all lesions seen on high-resolution CT scans while preserving healthy lung tissue.'

Fit to this patient. Likely ineligible The profile describes multiple small nodules dispersed through both lungs and states explicitly that the pattern is multiple and dispersed, not oligometastatic, which is the population the guideline carriage requires.

- blocker: Not oligometastatic: bilateral, multiple, dispersed pulmonary nodules per the profile

Standard in bone sarcoma only for oligometastatic lung disease, and the profile places this presentation outside that population on its face. Assessed and set aside so a reader who knows metastasectomy is sometimes curative in bone sarcoma can see it was weighed rather than missed; systemic control is the operative question for these lungs.

Alongside the targeted options. Independent of the targeted options.

Last verified. 2026-08-23

References. pmid:39550489